

POLYP DETECTION MEDICAL REPORT

Analysis Date: June 04, 2026

VIDEO INFORMATION

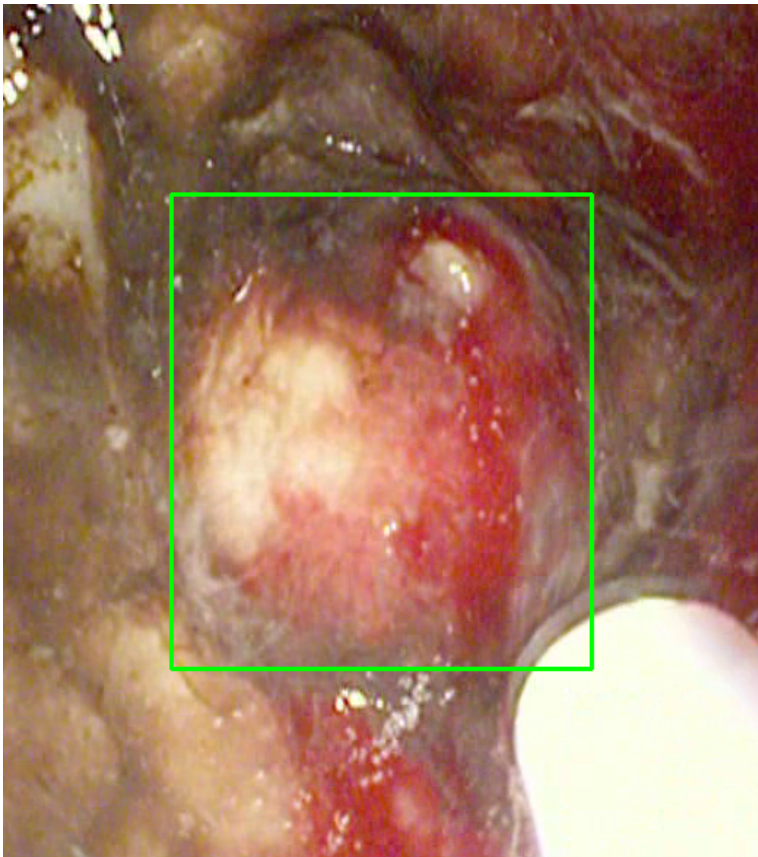
Video ID:	2eb34265-6359-4233-bdbb-71f3389e220a
Total Frames:	3006
Frame Rate:	25.0 fps (assumed)
Duration:	120.2 seconds

DETECTION SUMMARY

Detection Method	Count	Status
YOLO Detections	96	✓
RT-DETR Detections	941	✓
MedSAM2 Detections	942	✓
Consensus (3-Model Agreement)	5	✓
Risk Classification	Count	Percentage
HIGH RISK	0	0.0%
MEDIUM RISK	0	0.0%
LOW RISK	0	0.0%
TOTAL ANALYZED	5	100.0%

DETAILED POLYP ANALYSIS

POLYP #1 — FLAT_POLYP [MEDIUM_RISK] (Tracked across 315 frames)



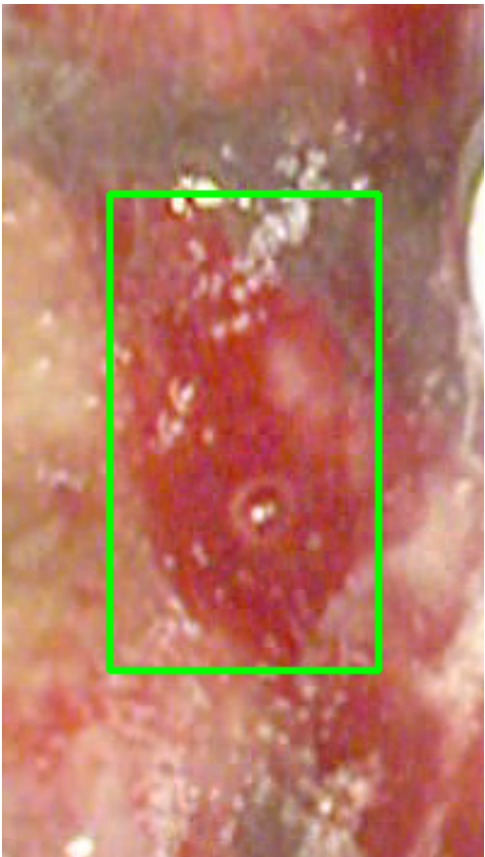
Feature	Value	Interpretation
Frame Index	1042	Frame 1042 of 3006
Detection Method	Detector Consensus (MedSAM2 + U-Net)	Model Agreement (MedSAM2 unavailable for this video)
POLYP TYPE	FLAT_POLYP	Type confidence: 90.9%
Redness Score	0.159	Color-based vascularization
Relative Radius	16.9%	Polyp size as % of frame diagonal
Texture Score	0.038	Surface morphology
Vessel Visibility	0.504	Vascularization indicator
Risk Tier	MEDIUM RISK	Validated by ESGE 2017 / NICE classification
Clinical Classification	HIGH_RISK	Final symbolic reasoning bucket

Expert Prediction	HIGH_RISK	Decision Tree output
Clinical Class	ADENOMATOUS_POLYP	Polypoid lesion — discrete mucosal protrusion identified
Detection Confidence	65.0%	YOLO / RT-DETR / track confidence

Type Assessment: Flat mucosal lesion — chromoendoscopy or NBI assessment recommended.

Low-confidence MODERATE RISK detection - Clinical review recommended

POLYP #2 — FLAT_POLYP [MEDIUM_RISK] (Tracked across 315 frames)



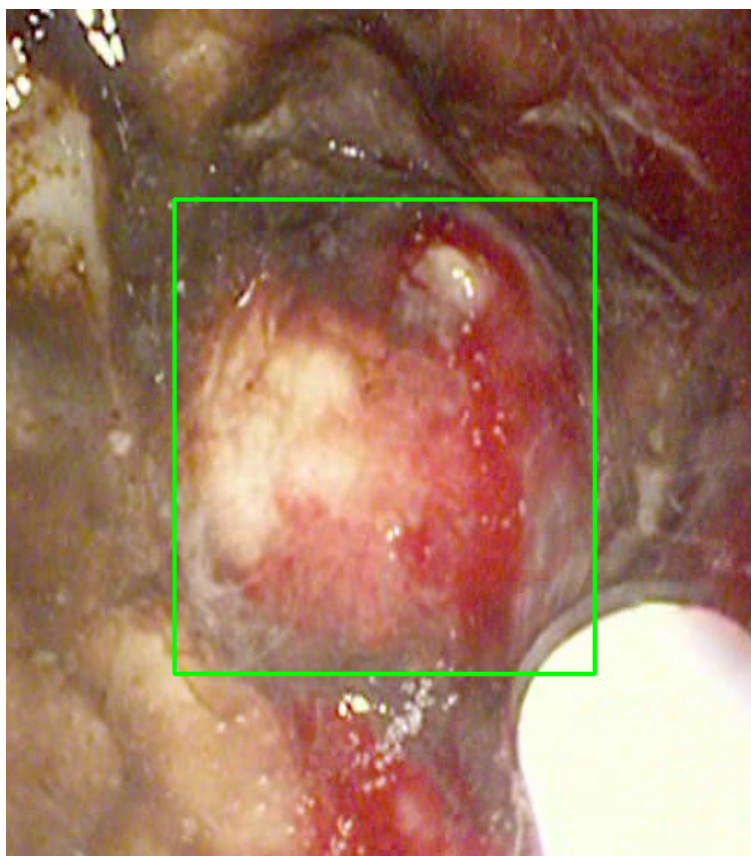
Feature	Value	Interpretation
Frame Index	1042	Frame 1042 of 3006
Detection Method	Detector Consensus (YOLO / RT-DETR)	Model Agreement (MedSAM2 unavailable for this video)
POLYP TYPE	FLAT_POLYP	Type confidence: 90.9%
Redness Score	0.159	Color-based vascularization

Relative Radius	16.9%	Polyp size as % of frame diagonal
Texture Score	0.038	Surface morphology
Vessel Visibility	0.504	Vascularization indicator
Risk Tier	MEDIUM RISK	Validated by ESGE 2017 / NICE classification
Clinical Classification	HIGH_RISK	Final symbolic reasoning bucket
Expert Prediction	HIGH_RISK	Decision Tree output
Clinical Class	ADENOMATOUS_POLYP	Polypoid lesion — discrete mucosal protrusion identified
Detection Confidence	59.9%	YOLO / RT-DETR / track confidence

Type Assessment: Flat mucosal lesion — chromoendoscopy or NBI assessment recommended.

Low-confidence MODERATE RISK detection - Clinical review recommended

POLYP #3 — BLEEDING_POLYP [HIGH_RISK] (Tracked across 148 frames)



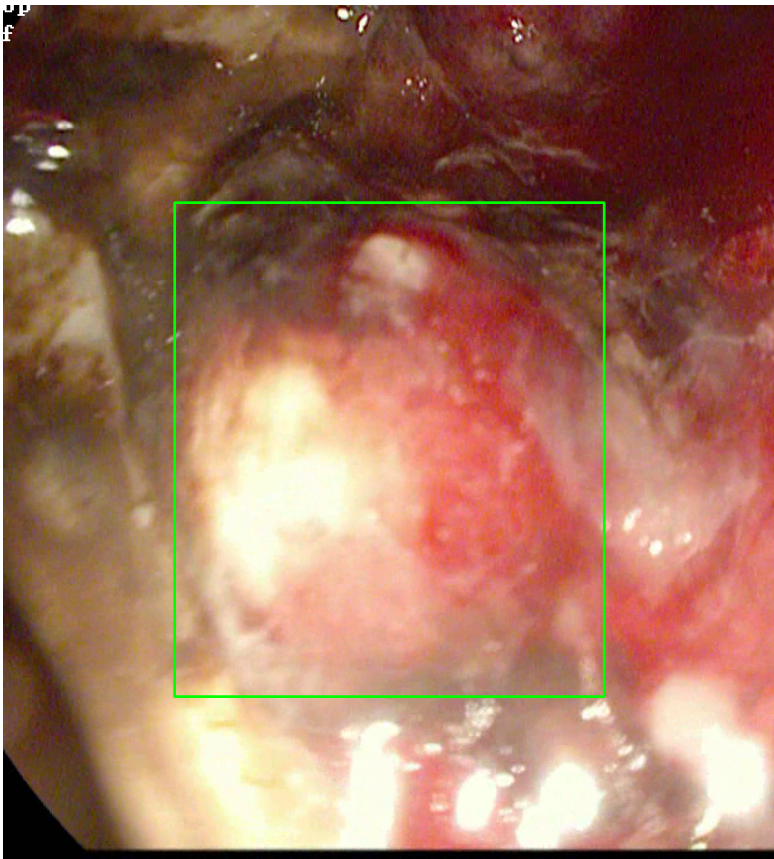
Feature	Value	Interpretation
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Frame Index	1042	Frame 1042 of 3006
Detection Method	Detector Consensus (MedSAM2 Unavailable)	MedSAM2 Unavailable (MedSAM2 unavailable for this video)
POLYP TYPE	BLEEDING_POLYP	Type confidence: 92.0%
Redness Score	0.281	Color-based vascularization
Relative Radius	11.2%	Polyp size as % of frame diagonal
Texture Score	0.055	Surface morphology
Vessel Visibility	0.953	Vascularization indicator
Risk Tier	HIGH RISK	Validated by ESGE 2017 / NICE classification
Clinical Classification	LOW_RISK	Final symbolic reasoning bucket
Expert Prediction	LOW_RISK	Decision Tree output
Clinical Class	ADENOMATOUS_POLYP	Polypoid lesion — discrete mucosal protrusion identified
Detection Confidence	61.8%	YOLO / RT-DETR / track confidence

Type Assessment: Active hemorrhagic lesion — elevated vascularity with redness signal. Immediate clinical review required.

Low-confidence HIGH RISK detection - Further evaluation recommended

POLYP #4 — SMALL_POLYP [LOW_RISK] (Tracked across 34 frames)



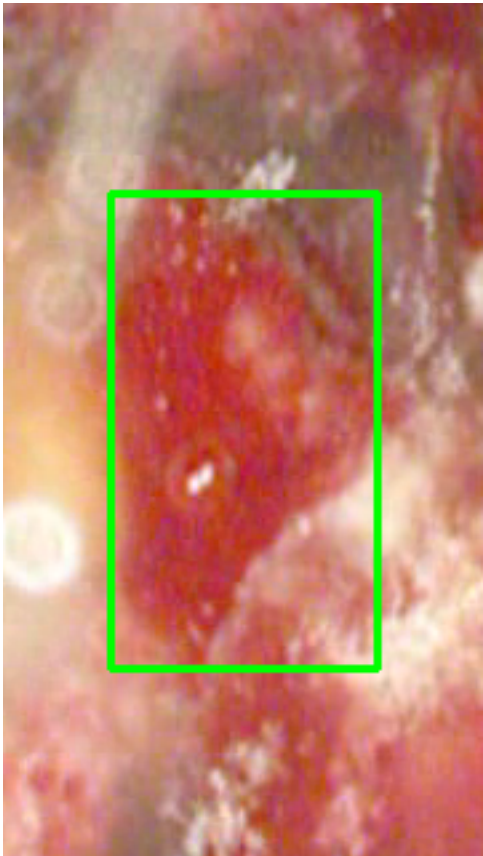
Feature	Value	Interpretation
Frame Index	1420	Frame 1420 of 3006
Detection Method	Detector Consensus (MedSAM2 Unavailable)	Detector Consensus (MedSAM2 Unavailable) (MedSAM2 unavailable for this video)
POLYP TYPE	SMALL_POLYP	Type confidence: 92.0%
Redness Score	0.232	Color-based vascularization
Relative Radius	6.6%	Polyp size as % of frame diagonal
Texture Score	0.023	Surface morphology
Vessel Visibility	0.389	Vascularization indicator
Risk Tier	LOW_RISK	Validated by ESGE 2017 / NICE classification
Clinical Classification	HIGH_RISK	Final symbolic reasoning bucket
Expert Prediction	HIGH_RISK	Decision Tree output

Clinical Class	ADENOMATOUS POLYP	Polypoid lesion — discrete mucosal protrusion identified
Detection Confidence	56.8%	YOLO / RT-DETR / track confidence

Type Assessment: Diminutive lesion — resect and discard per protocol.

Low-confidence LOW RISK detection - Follow standard protocol

POLYP #5 — BLEEDING_POLYP [HIGH_RISK] (Tracked across 111 frames)



Feature	Value	Interpretation
Frame Index	1288	Frame 1288 of 3006
Detection Method	Detector Consensus (YOLO / RT-DETR / MedSAM2 Unavailable)	Detector Consensus (YOLO / RT-DETR / MedSAM2 Unavailable) (MedSAM2 unavailable for this video)
POLYP TYPE	BLEEDING_POLYP	Type confidence: 92.0%
Redness Score	0.356	Color-based vascularization

Relative Radius	10.2%	Polyp size as % of frame diagonal
Texture Score	0.069	Surface morphology
Vessel Visibility	0.958	Vascularization indicator
Risk Tier	HIGH RISK	Validated by ESGE 2017 / NICE classification
Clinical Classification	HIGH_RISK	Final symbolic reasoning bucket
Expert Prediction	HIGH_RISK	Decision Tree output
Clinical Class	ADENOMATOUS_POLYP	Polypoid lesion — discrete mucosal protrusion identified
Detection Confidence	61.4%	YOLO / RT-DETR / track confidence

Type Assessment: Active hemorrhagic lesion — elevated vascularity with redness signal. Immediate clinical review required.

Low-confidence HIGH RISK detection - Further evaluation recommended

CLINICAL IMPRESSION

Summary:

A total of 5 polyps were detected and analyzed using multi-model consensus and AI-based risk stratification.

Risk Distribution:

- HIGH RISK polyps: 0 (0.0%)
- MEDIUM RISK polyps: 0 (0.0%)
- LOW RISK polyps: 0 (0.0%)

Recommendation:

All detected polyps are classified as LOW RISK. Standard surveillance protocol is recommended.

Technical Details:

- Detection: Multi-model consensus (YOLO, RT-DETR, MedSAM2)
- Features: 444-dimensional vectors (384 SSL + 60 biomarkers)
- Classification: Cluster-specific Decision Tree experts
- Confidence: Based on deep learning + medical heuristics

Disclaimer: This report is generated by an AI-assisted research system for academic and research purposes only. All clinical recommendations are derived from published ESGE 2017, NICE CG118/CG131, and BSG 2019 colonoscopy guidelines. This output does not constitute medical advice and must not be used as the sole basis for clinical decisions. All findings must be reviewed and confirmed by a qualified gastroenterologist or endoscopist.